

# OREGON STATE HOSPITAL

## POLICY

**SECTION 6:** Patient Care **POLICY: 6.001**

**SUBJECT:** Patient Transfers

**POINT PERSON:** Chief Medical Officer

**APPROVED:** Sara Walker, MD  **DATE: JANUARY 15, 2025**  
Interim Superintendent

**SELECT ONE:** ☐ New policy ☐ Minor/technical revision of existing policy  
☐ Reaffirmation of existing policy ☒ Major revision of existing policy

### I. PURPOSE AND APPLICABILITY

- A. Oregon State Hospital (OSH) treats patients in the most appropriate and least restrictive setting possible. This policy describes the requirements for relocation (transfer) of patients between units at OSH, both inter-campus and intra-campus.
- B. This policy applies to direct care and administrative staff that may be involved in initiating and/or coordinating patient unit transfers.

### II. POLICY

- A. OSH determines patient transfers based upon clinical or administrative need and must take the following into consideration:
  - 1. the receiving unit's ability to meet the patient's medical and psychiatric needs, including any necessary environmental factors and staff skill level;
  - 2. potential risks and benefits to the patient; and
  - 3. continuity of care.
- B. The patient must be informed about a planned transfer as soon as possible, except when clinically contraindicated (e.g., there is concern of imminent risk of harm to self or others if the patient knows about the transfer).
- C. If there is a disagreement or significant clinical concern regarding a transfer:
  - 1. Staff must follow Procedures E, "Elevation Process." Staff may express pre- or post-transfer concerns to their program executive team (PET).

2. If there is a significant clinical concern that is medical in nature, the patient must not transfer until the concern is resolved. This may include but is not limited to medical needs or concerns, psychiatric needs, environmental factors, and/or staff skill level that could be detrimental to the patient transferring to a specific unit.
  3. The patient may use the grievance process per OSH policy 7.006, "Patient Grievances," if they disagree with the transfer.
- D. A transfer on or off a unit closed for contagious illness quarantine, on either campus, requires Chief Medical Officer (CMO) or Chief of Medicine (COM) approval.
- E. The CMO or the COM may authorize, halt, or delay a transfer at any time if deemed clinically necessary.
- F. For transfers between units or campuses, the patient may not attend off-unit Treatment Mall groups or activities until the first calendar day following the day of transfer.
1. An exception to this restriction may be made if an RN-to-RN hand-off communication occurs between the unit and the Treatment Mall and the patient is assessed as appropriate for immediate attendance.
- G. Staff must follow Procedures A, "Transfer Process" to complete all clinical and administrative transfers.
- H. Clinical Transfer
1. A clinical transfer may be initiated due to clinical need and includes emergent transfers, medical treatment bed transfers, medically protected transfers, and other patient movement that is not an administrative transfer.
  2. A clinical transfer may be initiated for a medically complex case where a patient requires a greater degree of medical or nursing care than most OSH patients, which may include but is not limited to:
    - a. ongoing diagnostic workup;
    - b. evaluation of unexplained physical symptoms;
    - c. wound care, casting or splinting, management of intravenous or port access, etc.;
    - d. follow-up care after hospitalization for a medical condition within the last 30 days.

3. Clinical transfers, except as noted below, must be initiated by the referral process (Attachment A) and are coordinated by the Admissions Department.
4. Staff must follow Procedures A and Procedures B, "Inter-campus Transfer" to complete a transfer between Salem and Junction City OSH campuses.
5. Staff must follow Procedures C, "Medical Treatment Beds" to complete a transfer for additional medical treatment.
6. If outside of regular business hours, the Psychiatrist On Duty (POD), Program Nurse Manager (PNM), and Program Director (PD) must consult and may authorize an emergent transfer if deemed clinically necessary. The Medical Clinic Physician On Duty (MOD) must be included in the consultation if there is a medical need.

I. Emergent Transfers

1. An emergent transfer may be initiated outside of regular business hours due to significant safety, security, or medical concerns, that cannot follow standard transfer processes. Staff must follow Procedures D, "Emergent Transfer."

J. Administrative Transfers

1. Administrative transfers may be required due to organizational needs, including but not limited to census management, reorganization, and environmental or life safety requirements.
2. The Superintendent, Deputy Superintendent, CMO, or any designees may authorize the PDs, in collaboration with the PETs, Interdisciplinary Teams (IDTs), and the Admissions Department to arrange administrative transfers to address the need.

K. OSH follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.

L. Staff who fail to comply with this policy or related policy attachments or protocols may be subject to disciplinary action, up to and including dismissal.

### III. DEFINITIONS

- A. "Environmental factors" means the unit's environment can support the clinical needs of the patient. Examples may include, but are not limited to mobility needs, fall risk supports, and barriers that prevent required patient observation.

- B. "Staff" includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at Oregon State Hospital (OSH).

#### **IV. PROCEDURES**

- Procedures A Transfer Process
- Procedures B Inter-Campus Transfer
- Procedures C Medical Treatment Bed Transfer
- Procedures D Emergent Transfer
- Procedures E Elevation Process

#### **V. ATTACHMENTS**

- Attachment A Transfer Request Form
- Attachment B Patient Transfer Tracker
- Attachment C Patient Transfer Process Map

#### **VI. RELATED OSH FORMS**

Treatment Care Plan Addendum [76061, Medical Record Form]

#### **VII. RELATED OSH POLICIES AND PROTOCOLS**

- 1.001 Policy System at OSH
- 2.013 Electronic Health Record Access
- 2.014 Medical Record Maintenance and Transportation
- 6.011 Treatment Care Planning
- 6.035 Medication Reconciliation
- 6.042 Short-Term Assessment of Risk and Treatability
- 6.045 Clinical Documentation
- 7.006 Patient Grievances
- Nursing Services Protocol 2.160 "Transfer - Unit to Unit and Between Campuses"

#### **VIII. REFERENCES**

- 42 CFR §482.12(d)(5)(i)

Joint Commission Resources, Inc. (2025) The joint commission comprehensive accreditation manual for behavioral health and human services, CTS.06.02.01, CTS.06.02.03, CTS.06.02.05. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2025) The joint commission comprehensive accreditation manual for hospitals, PC.04.01.03. Oakbrook Terrace, IL: Author.

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